Fax

To: SANDIGO, GUSTAVO	**From:** Pam Health at Home - Texas
Fax: 13614850684	**Pages:** 10
Phone:	**Date:** 03-02-2026 5:26 PM
Re: Order(s) for Signature	

Comments:

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.
Thank you!

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

WARNING - PLEASE NOTE: The information transmitted herein is intended only for the person or entity to which it is addressed and may contain confidential, proprietary, protected, and/or privileged material. Any review, retransmission, disclosure, dissemination or other use of this information by persons or entities other than the intended recipient is prohibited. If you received this in error, please contact the sender and destroy this facsimile.

Order No: 485-26332

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
101643567600	02/26/2026	02/26/2026 - 04/26/2026	9730	10200500

6. Patient's Name and Address
Curtis, Roselie (9730)
409 Sequoia Dr
Victoria, TX 77904
Phone: (361) 655-5634

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 06/01/1941 | **9. Sex:** ☐ M ☒ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Amiodarone 200 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	200mg	daily	oral	N
Atorvastatin 40 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	40mg	daily	oral	LS
Calcium-500 500 mg (as calcium carbonate 1,250 mg) chewable tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	500mg	2 times a day	oral	LS
Centrum 18 mg-400 mcg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	1	daily	oral	LS
Cholecalciferol (vitamin D3) 125 mcg (5,000 unit) tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	1	daily	oral	LS
Coenzyme Q10 (ubiquinol) 200 mg capsule ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Capsule]	200mg	daily	oral	LS
Curcumin-phosphatidylcholine complex 500 mg capsule ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Capsule]	1	daily	oral	LS
Doxycycline hyclate 100 mg capsule ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Capsule]	100mg	2 times a day	oral	N
Eliquis 5 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	5mg	2 times a day	oral	LS
Furosemide 40 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	40mg	daily	oral	LS
Glucosamine Chondroit Complx Advan 750 mg-100 mg-125 mg-1.65 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	1	2 times a day	oral	LS

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Hydrocodone 10 mg-acetaminophen 325 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	1	3 times a day	oral	LS
Iron 325 mg (65 mg iron) tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	325mg	daily	oral	LS
Levothyroxine 50 mcg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	50mcg	daily	oral	LS
Lisinopril 2.5 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	2.5mg	daily	oral	LS
Melatonin ER 3 mg tablet,extended release ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	3mg	daily	oral	LS
Metoprolol succinate ER 50 mg tablet,extended release 24 hr ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	50mg	2 times a day	oral	LS
Restasis 0.05 % eye drops in a dropperette ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Metric Drop]	1	2 times a day	ophthalmic (eye)	LS
Sertraline 100 mg tablet ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	100mg	daily	oral	LS
Slow-Mag 71.5 mg tablet,delayed release ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Tablet]	1	2 times a day	oral	LS
Tolcylen 1 % topical solution ***As Prescribed by Physician*** [Amount Of/ Dosage Form: 1 Milliliter]	1	2 times a day	topical	LS

11. ICD-9 / 10 CM Principal Diagnosis

Code	Code Description	Date	O/E
T82.110D	Breakdown (mechanical) of cardiac electrode, subsequent encounter		

12. ICD-9 / 10 CM Surgical Procedure

Code	Code Description	Date	O/E

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13. ICD-9 / 10 CM Other Pertinent Diagnosis

Code	Code Description	Date	O/E
I48.0	Paroxysmal atrial fibrillation		
I11.0	Hypertensive heart disease with heart failure		
I50.9	Heart failure, unspecified		
E78.5	Hyperlipidemia, unspecified		
F32.A	Depression, unspecified		
E03.9	Hypothyroidism, unspecified		
Z95.0	Presence of cardiac pacemaker		
Z79.891	Long term (current) use of opiate analgesic		
Z79.02	Long term (current) use of antithrombotics/antiplatelets		
Z79.899	Other long term (current) drug therapy		
Z79.2	Long term (current) use of antibiotics		
Z79.01	Long term (current) use of anticoagulants		

14. DME and Supplies:
DME: Dressing Supplies, Grab Bars, Tub/Shower Bench, Walker

15. Safety Measures:
911 Emergency / Protocol, 911 for Emergency, Assistance with IADLs, Emergency Plan / Measures, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Fall Precautions, Hazard-Free Environment, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Keep Pathways Clear and Safe, Medication Safety / Storage, Safe Disposal of Sharp, Contaminated Materials, Safe in Movements, Safe Transfers, Safety in ADLs, Skin Care, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
Sodium Diet: low, No Added Salt,

17. Allergies:
Medication Allergies: flecainide acetate, ibandronate sodium, latex, lidocaine, morphine, procaine HCl, Sulfa (Sulfonamide Antibiotics)

18A. Functional Limitations:
☐ 1- Amputation ☐ 4- Hearing ☐ 7- Ambulation

18B. Activities Permitted:
☐ 1- Complete Bedrest ☒ 5- Exercises Prescribed ☐ 9- Cane

Medicare#:

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☐ 2- Bowel/Bladder (incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☑ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☐ 3- Fair ☑ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:

Frequency
SN Frequency: 1W1, 2W2, 1W2 Starting Week of 02/26/2026,

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Weight: < 188 or > 198
Pain: > 7

SN

ANTICOAGULANT THERAPY:
- Clinician will assess/instruct patient/caregiver on anticoagulant therapy, to include diet, safety, reportable s/s.
- SN to instruct patient/caregiver on medication side effects to report to SN or Physician.

~~**BEST PRACTICE FALL PREVENTION:**~~
- ~~Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.~~

~~**BEST PRACTICE MEDICATIONS/DRUG EDUCATION:**~~
- ~~Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.~~

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- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:

- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION:

- Clinician will perform validated test Braden to assess patient for risk of development of Pressure ulcers.
- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

CARDIOVASCULAR: CHF:

- Clinician to assess/instruct patient/caregiver Symptoms in Heart failure indicated by clinical heart failure guidelines (including dyspnea, orthopnea, edema, or weight gain).

CHF:

- SN to instruct pt/cg on possible s/s of CHF such as SOB, increased fatigue or weakness, edema in LE, chest pain. and weight gain of 3lbs/1 day and/or 5lbs/1week.
- SN to instruct pt/cg on ways to manage CHF such as taking appropriate medications as ordered, wearing compression stockings, avoiding excess salt,exercise as recommended by MD, and reporting to MD/SN weight gain of 3lbs/1 day and/or 5lbs/1week.
- SN to teach patient/cg importance of taking daily weights. Upon awakening each morning, and after using restroom, weigh yourself. Scale should be placed on hard surface. Record weight on monthly calendar. Report to MD/SN weight gain of 3lbs/1day and/or 5lbs/1week.
- SN to instruct on edema management

GENITOURINARY:

- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.
- Clinician to assess patient for presence of bowel and bladder incontinence. Instruct patient /caregiver measures to improve incontinence.

Instruct patient/caregiver on signs/symptoms of UTI to report to SN/MD.

SURGICAL WOUND:

- Clinician to assess surgical wound, instruct pt on s/s of infection to report-redness, warmth, edema, increased or sudden drainage, fever.

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: MALFUNCTION OF PACEMAKER S/P LEAD REVISION, AFIB, HTN, HLD, SLEEP APNEA, HYPOTHRYOID, CHF, DEPRESSION
- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition,

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exercise, etc.

• Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN:

• SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.

• SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.

• SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.

• SN to instruct patient/caregiver on Heart Healthy diet.

CARDIOVASCULAR - AFIB:

• SN to instruct pt/cg on s/s of Afib such as: weakness, rapid or irregular heartbeat, fluttering or thumping in the chest, dizziness, SOB, or lightheadedness

• SN to instruct pt/cg on how to manage Afib such as: eating a heart healthy diet, take medication as prescribed, avoid common triggers (infections, heart failure, alcohol, caffeine, skipping medications, smoking), maintaining a healthy BMI, and exercising as recommended by the doctor.

• SN to instruct pt/cg on s/s stroke and when to notify EMS

• SN to instruct patient/caregiver on high-risk medications anticoagulants, and antiplatelet medications as prescribed.

Assessment Summary

ADMIT of an 84 year olf female with a past medical history of-AFIB, HTN, HLD, SLEEP APNEA, HYPOTHRYOID, CHF, DEPRESSION. On 2/20/26 pt had a syncope episode and called EMS. Pt was taken to Citizens hospital where she was found to have a malfunctioning pacemaker. Pt was admitted and on 2/24/26 she underwent a lead revision surgery by/ Dr Scruggs. Pt was discharged home following procedure. Pt had medication changes-Stop Baclofen, added Amiodarone. Doxycycline 2xday for 7 days for prophylaxis. All medications in the home. Pt has family member staying with her to assist with adl and appt needs. Pt had follow up with pcp yesterday 2/25/26 with no changes. Pts surgical site is clean, dry, with steri strips in place. No s/s of infection present, no drainage. Pt was educated on s/s to monitor for and report-redness, drainage, fever, edema. Pt acknowledged s/s. Pt has a soft sling to wear during the night to left arm. Pt denies using assistive device, but does have a walker in the home. Due to recent surgery pt is at a higher fall risk from weakness. Pt is independent with medications. Pt is currently homebound due to-weakness, unsteady gait, fall risk, must be accompanied by another person in order to leave the home. Pt to be admitted to PAM HH for SN-assessments, disease process education and management, medication education, surgical site monitoring and education, fall prevention education, pain management education, pu prevention education. Pt goal is to heal without complications and transfers to out pt PT with PAM.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.

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May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. Curtis, Roselie is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 02.24.2026 BY SCHMIDT, DAVID ACNP and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 02.26.2026.

Risk of Hospitalization

Risks: Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications; Currently reports exhaustion;
Interventions: Referrals - SN; Immunizations - Influenza, Pneumonia; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...); Tele monitoring
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); Help with managing Medications needed (M2020); Lives Alone (M1100) ; Inadequate Support Network (M1100); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

Primary Language -ENGLISH;

Cognitive Status

Cognitive status: No behaviors demonstrated

Advance Directives

Living will; Copies on file; POA; Healthcare representative

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Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Unable to safely leave home unassisted;
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments:

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Good to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

SN

ANTICOAGULANT THERAPY: - LTG
- LTG-Patient will be free of adverse s/s of anticoagulant therapy throughout episode.

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG
- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

CARDIOVASCULAR: CHF: - LTG
- LTG-Patient/caregiver will receive education on Symptoms indicative of heart failure and will be able to recall S/s of Heart failure and when to report by end of episode.

CHF: - STG
- STG-Patient/caregiver will perform weights and maintain daily weight log by 4 weeks

GENITOURINARY: - LTG
- LTG-Patient will tolerate collection of urine without pain and discomfort throughout episode.

SURGICAL WOUND: - STG
- STG-Patient/Caregiver will be able to identify s/s of infection of surgical wound by 2weeks.

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG

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- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

CARDIOVASCULAR - AFIB: - LTG

- Patient/caregiver will be able to verbalize s/s of Atrial fibrillation, and strategies to manage Afib.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 02/26/2026
Signed By: Martinez, Cassandra - RN on 02/26/2026 04:25 PM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
Gustavo Sandigo MD
202 James Coleman Drive Ste A
VICTORIA, TX 77904,NPI: 1174571624, License#: J2777,Phone: (361) 573-4000, Fax: (361) 485-0684

26. I certify this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy, or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care, and will periodically review the plan. I further certify this patient had a face-to-face encounter that was performed by a physician or Medicare allowed non-physician practitioner that was related to the primary reason the patient requires home health services.

27. Attending Physician's Signature and Date Signed:

3/3/26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.